

UC Treatment Comparison Guide

Plain-English comparisons of every major UC medication class — so you can have informed conversations with your gastroenterologist.

Aminosalicylates (5-ASA)

Mesalamine, Sulfasalazine — first-line for mild-moderate UC

Corticosteroids

Prednisone, Budesonide — for active flares, not long-term use

Immunomodulators

Azathioprine, 6-MP — long-term maintenance option

Anti-TNF Biologics

Infliximab, Adalimumab, Golimumab — inject or infuse

Gut-Selective Biologic

Vedolizumab — targets the gut only, fewer systemic effects

IL-12/23 Biologic

Ustekinumab — injection after IV loading dose

JAK Inhibitors

Tofacitinib, Upadacitinib — oral pills, fast acting

MEDICAL DISCLAIMER: This guide is for educational purposes only. It does not constitute medical advice, diagnosis, or treatment. Never start, stop, or change a medication without guidance from your gastroenterologist. Individual treatment decisions depend on your full medical history.

Aminosalicylates and Corticosteroids

UC treatment follows a "step-up" approach. Most patients start with aminosalicylates (5-ASA). If symptoms are not controlled, treatment steps up to immunomodulators, then biologics or JAK inhibitors. Corticosteroids are used to control active flares — not as long-term maintenance.

AMINOSALICYLATES (5-ASA) — First-Line Therapy

Mesalamine (Lialda, Asacol, Pentasa, Delzicol)

FIRST-LINE

How taken: Oral tablets or capsules daily. Rectal suppositories or enemas for distal disease.
Used for: Mild to moderate UC — the starting point for most newly diagnosed patients.
How fast: 2 to 8 weeks for full anti-inflammatory effect.

Advantages:

- + Well tolerated by most patients with few serious side effects
- + Reduces relapse risk significantly when taken long-term and consistently
- + Generally considered safe during pregnancy (discuss with your doctor)
- + Multiple formulations: pills, suppositories, enemas — can target specific areas

Considerations:

- Must be taken every day even during remission — skipping causes flares
- Not effective for moderate-severe disease or extensive colitis
- Rectal forms may be needed alongside oral for distal disease

Ask your doctor: *Am I getting the most from my current mesalamine dose and formulation? Should I add rectal therapy?*

Sulfasalazine (Azulfidine)

ALTERNATIVE 5-ASA

How taken: Oral tablets, usually 2-4 times daily with food.
Used for: Mild to moderate UC. Also effective for UC-associated joint inflammation.
How fast: 4 to 8 weeks for full effect.

Advantages:

- + Lower cost than newer mesalamine formulations — often fully covered
- + Helps joint symptoms that sometimes accompany UC
- + Decades of safety data available

Considerations:

- More side effects than newer 5-ASA: nausea, headache, sensitivity to sun
- Must take folic acid supplement alongside it
- Can reversibly reduce sperm count in men — discuss if planning a family

Ask your doctor: *Would sulfasalazine be a better fit given my budget and joint symptoms?*

CORTICOSTEROIDS — Active Flare Control Only

Corticosteroids control flares but **DO NOT** maintain remission. Using them repeatedly is a signal that your maintenance treatment is not working. Discuss stepping up therapy with your doctor.

How taken:

Oral tablets (prednisone) or IV infusion (methylprednisolone) for hospitalized patients.

Used for:

Moderate to severe active UC flares. NOT appropriate for long-term maintenance.

How fast:

Days to 1-2 weeks for symptom relief.

Advantages:

- + Fast and effective at controlling active flares
- + Available oral and IV — adaptable to flare severity
- + Reduces inflammation quickly when other treatments need time to work

Considerations:

- Significant side effects with prolonged use: weight gain, mood changes, bone loss, blood sugar rise, sleep disruption
- Relapse is common when stopping — does not maintain remission
- Needing steroids more than once a year means stronger maintenance therapy is needed

Ask your doctor: *If I need steroids again this year, does that mean we need to change my maintenance medication?*

Budesonide MMX (Uceris)**LOWER-RISK STEROID**

How taken: Extended-release oral tablet once daily. Releases in the colon specifically.

Used for: Mild to moderate active UC. Fewer whole-body effects than prednisone.

How fast: 1 to 4 weeks.

Advantages:

- + Fewer systemic side effects — acts locally in the colon
- + Less impact on blood sugar, bone density, and mood than prednisone
- + Good option for patients who experience severe side effects from prednisone

Considerations:

- Less potent than prednisone — may not control severe flares
- Still not appropriate for long-term maintenance
- More expensive than generic prednisone

Ask your doctor: *Would budesonide be safer than prednisone given my other health conditions?*

Immunomodulators and Anti-TNF Biologics

IMMUNOMODULATORS — Long-Term Maintenance

Immunomodulators work slowly — they take 3 to 6 months to reach full effect. They are never used alone for an active flare. They are most often combined with a biologic to prevent the body from developing antibodies against it.

Azathioprine (Imuran) / 6-Mercaptopurine (6-MP)

IMMUNOMODULATOR

How taken: Oral tablet taken once daily.

Used for: Moderate UC maintenance. Often combined with a biologic medication.

How fast: Very slow — 3 to 6 months for full therapeutic effect.

Advantages:

- + Can maintain remission long-term once it takes effect
- + Lower cost than biologic medications
- + Reduces antibody formation when used alongside biologics (combination therapy)

Considerations:

- Cannot control an active flare — too slow
- Requires regular blood monitoring: complete blood count and liver function tests
- TPMT enzyme test required before starting to determine safe dosing
- Small increased infection risk with long-term use

Ask your doctor: *Should I be on an immunomodulator alongside my biologic to prevent antibody formation?*

ANTI-TNF BIOLOGICS — Moderate to Severe UC

Biologics are large-molecule medications made from living cells. They target specific proteins in the immune system. Anti-TNF biologics block Tumor Necrosis Factor-alpha, a key driver of gut inflammation in UC.

Infliximab (Remicade, Inflectra, Renflexis, Avsola)

ANTI-TNF BIOLOGIC

How taken: IV infusion at weeks 0, 2, and 6 (loading), then every 8 weeks for maintenance.

Used for: Moderate to severe UC. Also used for steroid-dependent or steroid-refractory disease.

How fast: 2 to 6 weeks for initial response. Full effect assessed at week 14.

Advantages:

- + One of the most studied biologics in UC with decades of real-world data
- + Used for both induction (bringing into remission) and maintenance
- + Biosimilar versions (Inflectra, Renflexis, Avsola) available at significantly lower cost
- + Can be used in hospitalized patients with severe flares via IV

Considerations:

- IV infusion required at a clinic or infusion center every 6-8 weeks
- Can develop antibodies over time, reducing effectiveness (combination with immunomodulator helps)
- TB screening, hepatitis B test, and infection screening required before starting
- Not for patients with certain infections, moderate-severe heart failure, or multiple sclerosis

Ask your doctor:

Is infliximab still the right biologic for me, or should we try a different target?



How taken:

Self-injected at home. Adalimumab: every 2 weeks. Golimumab: once monthly.

Used for:

Moderate to severe UC.

How fast:

4 to 12 weeks for meaningful clinical response.

Advantages:

- + Self-injection at home — no clinic visits required for infusions
- + Adalimumab widely available in biosimilar form at lower cost
- + Golimumab offers once-monthly dosing — convenient for some patients
- + Good option for patients who want independence from infusion scheduling

Considerations:

- Antibody formation possible — may lose effectiveness over time
- TB and infection screening required before starting
- Injection site reactions possible, especially at the start

Ask your doctor: *Would a self-injectable anti-TNF fit better with my schedule than a clinic infusion?*

Gut-Selective and Newer Biologic Therapies

GUT-SELECTIVE AND IL-12/23 BIOLOGICS

Vedolizumab (Entyvio)

GUT-SELECTIVE

How taken: IV infusion every 8 weeks (loading at 0, 2, 6 weeks). Subcutaneous injection form available.

Used for: Moderate to severe UC. Preferred when systemic infection risk is a concern.

How fast: Slower than anti-TNF — meaningful response at 6 to 14 weeks.

Advantages:

- + Gut-selective mechanism: acts only in the gastrointestinal tract, not the whole immune system
- + Lower risk of serious systemic infections compared to anti-TNF biologics
- + Good choice for patients with prior cancers, recurrent infections, or older patients
- + Subcutaneous injection form now available for home administration

Considerations:

- Slower onset than anti-TNF — not ideal if rapid response is needed
- Less effective for extraintestinal manifestations like joint pain or skin involvement
- IV infusion clinic visits required unless using the injection form

Ask your doctor: *Given my health history, is vedolizumab safer for me than an anti-TNF biologic?*

Ustekinumab (Stelara)

IL-12/23 BIOLOGIC

How taken: Single IV loading dose based on weight, then subcutaneous injection every 8 to 12 weeks.

Used for: Moderate to severe UC. Also approved for Crohn's disease.

How fast: 4 to 8 weeks after the IV loading dose for initial response.

Advantages:

- + Very convenient after loading: only 4 to 6 injections per year
- + Favorable safety profile — lower infection risk than anti-TNF
- + Effective for patients who have failed or lost response to anti-TNF biologics
- + Also covers skin and joint manifestations of UC

Considerations:

- Requires an IV infusion for the first loading dose
- Insurance prior authorization is often required and may take time
- Less data than older biologics, though the evidence base is growing rapidly

Ask your doctor: *Is ustekinumab a good next step after failing my current biologic?*

JAK INHIBITORS — Oral Small-Molecule Therapies

JAK inhibitors are oral pills — no injections or infusions. They work by blocking Janus kinase enzymes inside cells, reducing the inflammatory signals that drive UC. They generally work faster than biologics.

BLACK BOX WARNING (FDA): JAK inhibitors carry an increased risk of serious infections, blood clots, major cardiac events, and certain cancers — particularly at higher doses. They are not recommended for patients over 65, smokers, or those with cardiovascular risk factors unless no alternatives exist. Discuss your personal risk with your doctor.

How taken:

Oral tablet twice daily (induction). Extended-release once daily for maintenance.

Used for:

Moderate to severe UC. FDA-approved. Used after 5-ASA failure or biologic failure.

How fast:

Fast — clinical response often within 2 to 4 weeks.

Advantages:

- + Oral pill — no injections or infusions needed
- + Fast onset of action — one of the quickest UC treatments available
- + Effective for patients who have failed multiple biologics

Considerations:

- FDA black box warning for infections, blood clots, cardiovascular events, malignancy at high dose
- Not recommended for patients over 65 or with significant cardiovascular risk
- Requires monitoring: lipid panel, CBC, liver function tests
- Herpes zoster (shingles) vaccination recommended before starting

Ask your doctor: *Given my age and cardiovascular history, is tofacitinib safe and appropriate for me?*

Upadacitinib (Rinvoq)**JAK1-SELECTIVE**

How taken: Oral extended-release tablet once daily (higher induction dose, lower maintenance dose).

Used for: Moderate to severe UC. Also approved for Crohn's disease.

How fast: Fast — response often within 2 to 4 weeks of starting induction dose.

Advantages:

- + Once-daily oral dosing — simple regimen
- + More selective for JAK1 than tofacitinib — potentially more targeted
- + Strong remission rates even in patients who failed previous biologics
- + Approved for both UC and Crohn's disease

Considerations:

- Same FDA black box warnings as tofacitinib
- Acne is a common and frequently reported side effect
- Requires regular laboratory monitoring
- Insurance prior authorization required in most cases

Ask your doctor: *How does upadacitinib compare to biologics for my specific disease severity and history?*

Quick-Reference Comparison Table

Use this table as a conversation starter with your gastroenterologist — not as a guide to self-prescribe.

Medication Class	Route	Speed	Best Suited For	Key Watch-Out
Mesalamine (5-ASA)	Oral / Rectal	2-8 weeks	Mild-Moderate	Take every single day
Sulfasalazine	Oral	4-8 weeks	Mild-Mod + joints	Folic acid needed
Corticosteroids	Oral / IV	Days	Active flares only	Not long-term
Immunomodulators	Oral daily	3-6 months	Maintenance combo	Blood monitoring
Infliximab (anti-TNF)	IV infusion	2-6 weeks	Moderate-Severe	Antibody formation
Adalimumab (anti-TNF)	Injection	4-12 weeks	Moderate-Severe	Home injection
Vedolizumab	IV / Inject	6-14 weeks	Gut-targeted	Slower onset
Ustekinumab	IV then inject	4-8 weeks	Post-anti-TNF	Auth needed
Tofacitinib (JAK)	Oral daily	2-4 weeks	After biologic fail	Cardiac/clot risk
Upadacitinib (JAK)	Oral daily	2-4 weeks	Moderate-Severe	Cardiac/clot risk

IMPORTANT: The "right" medication depends on your disease extent, prior treatments, other health conditions, insurance coverage, and your personal preferences. This table is a starting point for an informed conversation — not a prescription guide.

Prior Authorization (PA): Most insurance plans require a PA for biologics and JAK inhibitors before they will cover the cost. Your GI office handles the paperwork — but ask them how long it typically takes so you can plan your treatment timeline.

Questions to bring to your next appointment:

- Is my current treatment controlling my UC — or is my disease progressing?
- Am I a candidate for a biologic or JAK inhibitor based on my disease severity?
- What treatment would you recommend for someone with my exact history?
- How do we measure success — and how will I know if it is working?

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More free resources at **colitishelpusa.com**

UC Flare Survival Guide (Free PDF)

colitishelpusa.com/free-uc-flare-guide

25 Questions to Ask Your GI Doctor (Checklist)

colitishelpusa.com/gi-doctor-questions

Biologics for UC — Full Plain-English Guide

colitishelpusa.com/blog/biologics-for-ulcerative-colitis

JAK Inhibitors for UC

colitishelpusa.com/blog/jak-inhibitors-ulcerative-colitis

UC Insurance and Cost Coverage Guide

colitishelpusa.com/ulcerative-colitis-insurance-coverage

Free UC Care Options Check (8 questions)

colitishelpusa.com/uc-care-options-check

a licensed gastroenterologist for decisions about your UC care.